

**ASPRIL TABLET 300MG**

Description: Yellow, round, flat, top-scored tablet. Diameter = 10mm

Content:

Acetyl salicylic acid 300.0mg/tab.

Indication:

For the relief of headache, fever, and other mild, moderate pain associated with cold & flu, toothache, treatment of acute and chronic rheumatic state and other non-rheumatic inflammatory conditions.

Pharmacology:

Acetylsalicylic acid has analgesic, anti-inflammatory and antipyretic actions, due to inhibition of the biosynthesis of prostaglandins. It is often described as peripherally acting compounds. Antipyretic activity is considered to involve the hypothalamus. Acetylsalicylic acid, affects platelet function, by blocking synthesis of thromboxane A₂. High doses may have the opposite effect since they can block the synthesis of prostacyclin, which is inhibits platelet aggregation. Absorption of non-ionized acetylsalicylic acid occurs in the stomach. Acetylsalicylates and salicylates are also readily absorbed from the intestine. Absorption is delayed by food and impaired in patients with migraine attacks, achlorhydria and following administration of polysorbates. In those also taking antacids, absorption takes place more rapidly. Some hydrolysis to salicylate occurs before absorption. Plasma half-life for acetylsalicylic acid about 17 minutes and for salicylate, 2-4 hours for low doses and up to 19 hours for high doses. Acetylsalicylic acid is bound to a small extent to plasma proteins. Rapid hydrolysis to salicylic acid, occurring mainly in the blood, along with glucuronic acid and glycine conjugation to form acyl and ether glucuronides and salicyluronic acid, a small fraction is oxidized to form gentisic acid, 2, 3, 5-trihydroxybenzoic acid and 2, 3-dihydroxybenzoic acid. Serum esterases concentration may be lower in females than in males and this may account for the higher incidence of gastric ulceration in women. The rate of excretion of acetylsalicylic acid varies with the pH of the urine, increasing as the pH rises and being greatest at pH 7.5 and above. In alkaline urine, about 85% of a dose is excreted in this form. It is completely excreted in the urine with about 80% of a dose excreted as salicyluronic acid, 10% as salicyl-o-glucuronic, 5% as salicyl ester glucuronide and 5-10% as free salicylic acid. Small amounts may additionally be excreted as gentisic acid or as unchanged drug.

Contraindications:

It is contraindicated in patients with peptic ulcer, asthma, hemophilia or other bleeding disorders, those sensitive to acetylsalicylic acid and during pregnancy.

Side effects / Adverse reactions:

The most common side effects are gastrointestinal disturbances (nausea, dyspepsia & vomiting). Irritation of the gastric mucosa with erosion, ulceration, haemetemesis and melaena may occur. Slight blood loss may occur in 70% of patients with most acetylsalicylic preparations buffered, soluble or plain and is not accompanied by dyspepsia. Acetylsalicylic acid produces reactions in hypersensitive patients, skin eruptions and swelling of the mucous membranes of the nose and throat with asthma-like attack. It may give rise to angioneurotic edema, urticaria and myocarditis. It decreases platelet adhesiveness and increases the coagulation time of the blood. Infants of mothers who have taken large doses regularly during pregnancy or while breastfeeding may have impaired platelet functions.

Precautions:

Special precautions for those with asthma, peptic ulcer, impaired renal/hepatic functions & pregnancy. Effects of acetylsalicylic acid on gastrointestinal tract is enhanced by alcohol. Not recommended for use in children <16 years.

Warning:**Risk of GI Ulceration, Bleeding and Perforation with NSAID**

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Drug Interactions:

The use of acetylsalicylic acid with anticoagulant drugs should be avoided as it may increase the risk of hemorrhage. Acetylsalicylic acid may increase the toxic effects of methotrexate and decrease the uricosuric of probenecid.

Dosage and administration:

Oral, 1 - 2 tablets three times daily after food.

Treatment of over dosage:

Symptoms: dizziness, deafness, sweating, nausea and vomiting, headache, mental confusion, hyperventilation, fever, restlessness, ketosis, respiratory alkalosis and metabolic acidosis. Depression of the central nervous system may lead to coma, cardiovascular collapse and respiratory failure. In children, drowsiness and metabolic acidosis commonly occur, hypoglycemia may be severe. **Treatment:** The stomach should be emptied by inducing emesis or by aspiration and lavage. Patients with mild intoxication should be encouraged to drink plenty of fluids. In patients with more severe intoxication (plasma salicylate concentration above 500ug/ml in adults or 300ug/ml in children), forced alkaline diuresis may be required and continued until the plasma concentration is less than 350ug/ml in adults, then intravenous fluid can be stopped and the patient is encouraged to take fluid by mouth. Intravenous of sodium chloride with sodium bicarbonate may be required to correct dehydration and metabolic disturbances.

Storage conditions: Keep container tightly closed. Store in a dry place below 30°C.

Protect from light.

Keep away from children.

Presentation

: 1000's/bot (EXPORT)

Shelflife

: 2 years

Manufactured & Distributed by

: PRIME PHARMACEUTICAL SDN. BHD.

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